LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Mr Lanclot**		
DOB/Age/Gender	: 33 Y 8 M 20 D/Male	Report STATUS	: Final Report
Patient ID / UHID	: 13281029/RCL12828942	Barcode NO	: 25747026
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Jul 29, 2025, 06:24 AM	Report Date	: Jul 29, 2025, 02:25 PM.

ilac-MRA MC-6353

CM-HC04

Complete Blood Count (CBC)

Test Description	Value(s)	Unit(s)	Reference Range
RBC Parameters			
Hemoglobin *Spectrophotometry*	**12.6**	g/dL	13.0 - 17.0
RBC Count *Electrical impedance*	4.5	10^6/µl	4.5 - 5.5
PCV *Calculated*	**38.1**	%	40 - 50
MCV *Calculated*	84.9	fl	83 - 101
MCH *Calculated*	28.1	pg	27 - 32
MCHC *Calculated*	33.1	g/dL	31.5 - 34.5
RDW (CV) *Calculated*	**15.6**	%	11.6 - 14.0
RDW-SD *Calculated*	**44.1**	fl	35.1 - 43.9
WBC Parameters			
TLC *Electrical impedance and microscopy*	7.5	10^3/µl	4 - 10
Differential Leucocyte Count			
Neutrophils *Flow-cytometry DHSS*	71.7	%	40-80
Lymphocytes *Flow-cytometry DHSS*	**19.6**	%	20-40
Monocytes *Flow-cytometry DHSS*	6.4	%	2-10
Eosinophils *Flow-cytometry DHSS*	2.2	%	1-6
Basophils *Flow-cytometry DHSS*	0.1	%	<2
Absolute Leukocyte Counts ***calculated***			
Neutrophils.	5.38	10^3/µl	2 - 7
Lymphocytes. *Calculated*	1.47	10^3/µl	1 - 3
Monocytes. *Calculated*	0.48	10^3/µl	0.2 - 1.0
Eosinophils. *Calculated*	0.17	10^3/µl	0.02 - 0.5
Basophils.	**0.01**	10^3/µl	0.02 - 0.5

Dr. Anita Shobha Flynn
Consultant Pathologist

LABORATORY REPORT

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Sample Collected	: Jul 29, 2025, 06:24 AM	Report Date	: Jul 29, 2025, 02:25 PM.

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Test Description	Value(s)	Unit(s)	Reference Range
Calculated			
Platelet Parameters			
Platelet Count *Electrical impedance and microscopy*	205	10^3/µl	150 - 410
Mean Platelet Volume (MPV) *Calculated*	9.5	fL	9.3 - 12.1
PCT *Calculated*	0.2	%	0.17 - 0.32
PDW *Calculated*	17	fL	8.3 - 25.0
P-LCR *Calculated*	32.5	%	18 - 50
P-LCC *Calculated*	67	10^9/L	44 - 140
Mentzer Index *Calculated*	18.87	%	> 13

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Jul 29, 2025, 06:24 AM	Report Date	: Jul 29, 2025, 03:59 PM.

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Test Description	Value(s)	Unit(s)	Reference Range

Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate *MODIFIED WESTERGREN*	**22**	mm/hr	0 - 10

Interpretation:
ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc.

Reference- Dacie and lewis practical hematology

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Consultant Pathologist

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Patient ID / UHID	: 13281029/RCL12828942	Barcode NO	: 25747026
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Jul 29, 2025, 06:24 AM	Report Date	: Jul 29, 2025, 07:45 PM.

ilac-MRA MC-6353

Test Description	Value(s)	Unit(s)	Reference Range

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) *HPLC*	5.3	%	<5.7
Estimated Average Glucose *calculated.*	105.41	mg/dL	-

Interpretation:
Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:
1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :
HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

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Patient NAME	: **Mr Lanclot**		
DOB/Age/Gender	: 33 Y 8 M 20 D/Male	Report STATUS	: Final Report
Patient ID / UHID	: 13281029/RCL12828942	Barcode NO	: 19686769
Referred BY	: Self	Sample Type	: FLUORIDE F
Sample Collected	: Jul 29, 2025, 06:24 AM	Report Date	: Jul 29, 2025, 02:25 PM.

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Test Description	Value(s)	Unit(s)	Reference Range

Glucose Fasting

Glucose Fasting *Hexokinase*	90	mg/dL	70 - 100

Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥126

Reference : American Diabetes Association

Comment :
Blood glucose determinations in commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

Note
1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus.
2.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

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Patient NAME	: **Mr Lanclot**		
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Patient ID / UHID	: 13281029/RCL12828942	Barcode NO	: 25937319
Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Jul 29, 2025, 06:24 AM	Report Date	: Jul 29, 2025, 04:44 PM.

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Test Description	Value(s)	Unit(s)	Reference Range

Creatinine

Creatinine *kinetic alkaline picrate*	**0.55**	mg/dL	0.72 - 1.25
eGFR (CKD-EPI)	133.58	ml/min/1.73 sq m	Normal Or High: >= 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15

Interpretation:
Creatinine estimation is done to assess kidney function. It is not dependent on dietary factors. Normal values are obtained in kidney diseases, except in advanced renal failure and therefore its estimation is more valuable if coupled with clearance.

"eGFR test is applicable for patients aged 18 years or more."

Uric Acid

Uric Acid *Uricase*	**2.6**	mg/dL	3.5 - 7.2

Interpretation:
Serum uric acid levels are very labile and show day to day and seasonal variation in some people. Levels are also increased by emotional stress, total fasting and increased body weight. Serum uric acid levels are used to diagnose and monitor treatment of gout, monitor chemotherapeutic treatment of neoplasms to avoid renal urate deposition with possible renal failure.

SGOT / AST

SGOT/AST *Enzymatic {NADH (without P5P)}*	22	U/L	5 - 34

Interpretation:
Serum AST is used for differential diagnosis of diseases of hepatobiliary system and pancreas. Increased values are seen in liver diseases like acute viral hepatitis, cirrhosis, biliary obstruction, primary or metastatic cancer, granuloma, hepatic ischaemia.

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Referred BY	: Self	Sample Type	: Serum
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Test Description	Value(s)	Unit(s)	Reference Range

SGPT / ALT

SGPT/ALT *Enzymatic {NADH (without P5P)}*	12	U/L	0 to 55

Interpretation:
Serum ALT is used for differential diagnosis of diseases of hepatobiliary system and pancreas. Increased in alcohalic hepatitis, cirrhosis, hepatocellular carcinoma, chronic hepatitis. Decreased in genito-urinary tract infection, malignancy, pyridoxal phosphate deficiency states (malnutrition, pregnancy, alcoholic liver disease).

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Test Description	Value(s)	Unit(s)	Reference Range

Lipid Profile

Total Cholesterol *enzymatic CHOD-PAP*	133	mg/dL	<200
Triglycerides *Glycerol phosphate oxidase*	130	mg/dL	<150
HDL Cholesterol *cholesterol oxidase and peroxidase*	**25**	mg/dL	>40
Non HDL Cholesterol *Calculated*	108	mg/dL	<130
LDL Cholesterol *Calculated*	82	mg/dL	<100
V.L.D.L Cholesterol *Calculated*	26	mg/dL	< 30
Chol/HDL Ratio *Calculated*	**5.32**	Ratio	3.5 - 5.0
HDL/ LDL Ratio *Calculated*	**0.3**	Ratio	0.5 - 3.0
LDL/HDL Ratio *Calculated*	3.28	Ratio	-

Interpretation:
Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence

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Test Description	Value(s)	Unit(s)	Reference Range

High Risk	of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age >/=45 years in Males & >/= 55 years in Females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

*** After an adequate non-pharmacological intervention for at least 3 months.**

References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.

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Test Description	Value(s)	Unit(s)	Reference Range

Vitamin D 25 Hydroxy

Vitamin D 25 - Hydroxy *CMIA*	**9.6**	ng/mL	Deficiency:<10ng/ml Insufficient:10-30ng/ml Sufficient:>30-100ng/ml Hypervitaminosis:>100ng/ml

Interpretation:
25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

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Referred BY	: Self	Sample Type	: Serum
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Test Description	Value(s)	Unit(s)	Reference Range

Thyroid Profile Total

Triiodothyronine (T3) *CMIA*	50.1	ng/dL	35 - 193
Total Thyroxine (T4) *CMIA*	7.33	µg/dL	4.87 - 11.2
Thyroid Stimulating Hormone (Ultrasensitive) *CMIA*	2.63	µIU/mL	0.35 - 4.94

Interpretation:

Pregnancy	Refrence Range TSH
1st Trimester	0 .1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Clinical Use:
1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance
Decreased TSH: Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. Primary Hypothyroidism: High TSH levels.
2. Secondary/Tertiary Hypothyroidism: Low TSH levels.
3. Euthyroid Sick Syndrome: Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Nomral	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

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Sample Collected	: Jul 29, 2025, 06:24 AM	Report Date	: Jul 29, 2025, 04:44 PM.

Test Description	Value(s)	Unit(s)	Reference Range

Estimated Glomerular Filtration Rate (eGFR)

Creatinine *kinetic alkaline picrate*	**0.55**	mg/dL	0.72 - 1.25
eGFR (CKD-EPI)	133.58	ml/min/1.73 sq m	Normal Or High: >= 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15

Interpretation:

1. The CKD-EPI equation, expressed as a single equation, is:

$GFR = 141 * \min(Scr/\kappa,1)^{\alpha} * \max(Scr/\kappa, 1)^{-1.209} * 0.993^{Age} * 1.018$ [if female] * 1.159 [if black]

Scr is serum creatinine (mg/dL), κ is 0.7 for females and 0.9 for males, α is -0.329 for females and -0.411 for males, min indicates the minimum of Scr/κ or 1, and max indicates the maximum of Scr/κ or 1.

2. The CKD-EPI (Chronic Kidney Disease Epidemiology Collaboration) equation was developed in an effort to create a more precise formula to estimate glomerular filtration rate (GFR) from serum creatinine and other readily available clinical parameters, especially at when actual GFR is >60 mL/min per $1.73m^2$.

Reference:

Levey et al. Annals of Internal Medicine 2009 May 5, 150 (9): 604-12

"eGFR test is applicable for patients aged 18 years or more."

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Test Description	Value(s)	Unit(s)	Reference Range
Electrolytes (Na/K/Cl)			
Sodium *Ion selective Electrode-Indirect.*	140	mmol/L	136 - 145
Potassium *Ion selective Electrode-Indirect.*	3.9	mmol/L	3.5 - 5.1
Chloride *Potentiometric*	107	mmol/L	98 - 107

Interpretation:
An electrolyte panel, which typically includes measurements of sodium (Na), potassium (K), and chloride (Cl) levels, is a common blood test that provides information about your body's electrolyte balance.

1. Sodium (Na):
1. Sodium is crucial in maintaining fluid balance in your body and is essential for nerve function and muscle contraction.
2. High sodium levels (hypernatremia) or low sodium levels (hyponatremia) can indicate various health conditions, including dehydration, kidney problems, or hormonal imbalances.

2. Potassium (K):
1. Potassium is vital for proper muscle function, including the heart muscle and nerve function, as well as maintaining fluid and electrolyte balance.
2. Abnormal potassium levels (hyperkalemia or hypokalemia) can indicate kidney dysfunction, dehydration, certain medications, or other underlying health issues.

3. Chloride (Cl):
1. Chloride works closely with sodium and potassium to maintain fluid balance and proper pH levels in the body.
2. Abnormal chloride levels may occur alongside imbalances in sodium and potassium and can indicate dehydration, kidney problems, respiratory issues, or metabolic disorders.

Healthcare providers typically order an electrolyte panel to evaluate and monitor electrolyte imbalances, diagnose certain medical conditions, assess kidney function, or monitor treatment effectiveness. It's usually part of routine blood testing, especially in cases of dehydration, kidney disease, heart conditions, or electrolyte disorders.

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Patient NAME	**: Mr Lanclot**		
DOB/Age/Gender	: 33 Y 8 M 20 D/Male	Report STATUS	: Final Report
Patient ID / UHID	: 13281029/RCL12828942	Barcode NO	: 27624270
Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: Jul 29, 2025, 06:20 AM	Report Date	: Jul 29, 2025, 04:52 PM.

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Urine Routine and Microscopic Examination

Test Description	Value(s)	Unit(s)	Reference Range
Physical Examination			
Volume *visual*	20	mL	
Colour *visual*	Pale Yellow		Pale yellow
Transparency *visual*	Clear		Clear
Deposit *visual*	Absent		Absent
Chemical Examination			
Reaction (pH) *Double Indicator*	6.0		5.5-8.0
Specific Gravity *Ion Exchange.*	1.010	0	1.010 - 1.030
Urine Glucose (sugar) *Oxidase / Peroxidase*	Negative		Negative
Urine Protein (Albumin) *bromophenol blue*	Negative		Negative
Urine Ketones (Acetone) *Legals Test*	Negative		Negative
Blood *Peroxidase Hemoglobin*	Negative		Negative
Leucocyte esterase *amino acid aster*	Negative		Negative
Bilirubin Urine *Diazotized dicholoroaniline*	Negative		Negative
Nitrite *Griless Test*	Negative		Negative
Urobilinogen *Ehrlichs Test*	Normal		Normal
Microscopic Examination			
Pus Cells (WBCs) *WET MOUNT*	3-4	/hpf	0-5
Epithelial Cells *WET MOUNT*	1-2	/hpf	0-4
Red blood Cells *WET MOUNT*	Absent	/hpf	Absent
Crystals *WET MOUNT*	Absent		Absent
Cast *WET MOUNT*	Absent		Absent
Yeast Cells *WET MOUNT*	Absent		Absent

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Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: Jul 29, 2025, 06:20 AM	Report Date	: Jul 29, 2025, 04:52 PM.

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Test Description	Value(s)	Unit(s)	Reference Range
Amorphous deposits *WET MOUNT*	Absent		Absent
Bacteria *WET MOUNT*	Absent		Absent
Protozoa *WET MOUNT*	Absent		Absent

Interpretation:

URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.

Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.

Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.

Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.

Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.

Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.

pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.

Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.

Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.

Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.

*** End Of Report ***

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